

1. Administrative & Study Identification

Full Study Title: Tumor-agnostic Precision Immuno-oncology and Somatic Targeting Rational for You (TAPISTRY) Phase II Platform Trial **Short Title/Acronym:** TAPISTRY Platform Study **Protocol Number:** B041932 **NCT Number:** NCT04589845 **Posting ID:** 3397838386935247 **Sponsor Name:** Roche (Hoffmann-La Roche) **Trial Status:** RECRUITING **Investigational Product:** Targeted therapies and immunotherapies tailored to oncogenotype (e.g., Entrectinib, Atezolizumab, Pralsetinib, Idasanutlin, Camonsertib, Trastuzumab emtansine [ATC Code: L01FD03]) **Phase of Development:** Phase II **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy (confirmed Objective Response Rate [ORR] as assessed by an Independent Review Committee [IRC]) of targeted therapies or immunotherapy, as single agents or in rational, specified combinations, in patients with advanced solid tumors harboring specific oncogenic genomic alterations or who are Tumor Mutational Burden (TMB)-high. **Secondary Objectives:** To evaluate investigator-assessed ORR, Duration of Response (DOR), Clinical Benefit Rate (CBR), Progression-Free Survival (PFS), time to Central Nervous System (CNS) progression, Overall Survival (OS), patient-reported outcomes, safety, and pharmacokinetics/immunogenicity.

2.2 Background & Rationale Actionable genomic alterations are found in many solid tumors across pediatric and adult populations. Identifying such alterations can match patients to genome-driven therapies. Although some therapies have tumor-agnostic approval, similar approvals remain an unmet need for other genome-driven cancers. TAPISTRY is a platform master protocol that leverages comprehensive next-generation sequencing (NGS) as a pragmatic means to evaluate multiple therapies in rare biomarker-selected populations. (Preferred UMLS Concept: Precision Medicine).

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional (Platform Umbrella Trial) **Control Type:** Single-arm (Multi-cohort assignments) **Blinding:** Open-label **Randomization:** N/A (Participants are assigned to specific cohorts based on NGS genetic alteration results)

3.2 Planned Enrollment & Duration Target **\$N\$:** 920 evaluable patients **Number of Sites:** 100+ global sites **Estimated Study Start:** Q4 2020 **Estimated Primary Completion:** Event-driven, depending on accrual rates for rare cohorts

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histologically or cytologically confirmed diagnosis of advanced and unresectable or metastatic solid malignancy. 2. Measurable disease as defined by RECIST v1.1, RANO (Response Assessment in Neuro-Oncology), or INRC (International Neuroblastoma Response Criteria). 3. Performance status as follows: Participants aged ≥ 18 years: ECOG Performance Status 0-2; Participants aged 16 to < 18 years: Karnofsky score $\geq 50\%$; Participants aged < 16 years: Lansky score $\geq 50\%$. 4. For participants aged ≥ 18 and < 18 years: adequate hematologic and end-organ function. 5. Disease progression on prior treatment, or previously untreated disease with no available acceptable treatment. 6. Adequate recovery from most recent systemic or local treatment for cancer. 7. Tumors determined to harbor specific oncogenic genomic alterations or who are TMB-high as identified by a validated NGS assay. 8. Life expectancy ≥ 8 weeks. 9. Ability to comply with the study protocol, in the investigator's judgment. 10. For female participants of childbearing potential: Negative serum pregnancy test ≤ 14 days prior to initiating study treatment, agreement to remain abstinent or use single or combined contraception methods that result in a failure rate of $< 1\%$ per year for the period defined in the cohort-specific inclusion criteria; and agreement to refrain from donating eggs during the same period. 11. For male participants: Willingness to remain abstinent or use acceptable methods of contraception as defined in the cohort-specific inclusion criteria. *(Note: Participants must also meet all cohort-specific inclusion criteria).*

4.2 Exclusion Criteria 1. Current participation or enrollment in another therapeutic clinical trial. 2. Any anticancer treatment within 2 weeks or 5 half-lives prior to the start of study treatment. 3. Whole brain radiotherapy within 14 days prior to start of study treatment. 4. Stereotactic radiosurgery within 7 days prior to start of study treatment. 5. Pregnant or breastfeeding, or intending to become pregnant during the study. 6. History of or concurrent serious medical condition or abnormality in clinical laboratory tests that, in the investigator's judgment, precludes the participant's safe participation in and completion of the study or confounds the ability to interpret data from the study. 7. Incomplete recovery from any surgery prior to the start of study treatment that would interfere with the determination of safety or efficacy of study treatment. 8. Significant cardiovascular disease, such as New York Heart Association cardiac disease (Class II or higher), myocardial infarction, or cerebrovascular accident within 3

months prior to enrollment, unstable arrhythmias, or unstable angina. 9. History of another active cancer within 5 years prior to screening that may interfere with the determination of safety or efficacy of study treatment with respect to the qualifying solid tumor malignancy. *(Note: Participants must not meet any cohort-specific exclusion criteria).*

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Assigned drug or drug regimen tailored to the NGS assay results at screening (e.g., Atezolizumab 1,200 mg every 21 days for adults in the TMB-high cohort). **Route of Administration:** Oral or Intravenous (IV), dictated by the cohort-specific assigned therapy. **Duration of Treatment:** Treatment continues until disease progression, loss of clinical benefit, unacceptable toxicity, participant/physician decision to discontinue, or death.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive medications allowed as specified by individual cohort protocols. **Prohibited:** Concurrent participation in other therapeutic clinical trials and concurrent receipt of unapproved systemic anticancer therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	Up to Day 0	Informed Consent, NGS Biomarker Testing (tissue/blood), Baseline Tumor Assessments (CT/MRI/PET)
2-12	Every 6-8 Weeks	Treatment Cycles: Drug Administration, Pharmacokinetic/Immunogenicity Blood Draws, Safety Labs; Tumor Assessment: Imaging assessments for efficacy (RECIST v1.1 / RANO / INRC)
13	End of Treatment	End of Treatment assessment, Survival and progression follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Confirmed Objective Response Rate (ORR) (confirmed response indicates ≥ 4 weeks after initial documentation of response). **Measurement Method:** Assessed by an Independent Review Committee (IRC) according to RECIST v1.1, RANO, or INRC.

7.2 Secondary & Exploratory Endpoints 1. IRC-assessed Duration of Response (DOR) per RECIST v1.1. 2. IRC-assessed Clinical Benefit Rate (CBR) per RECIST v1.1. 3. IRC-assessed Progression-Free Survival (PFS) per RECIST v1.1. 4. Overall Survival (OS) and Time to CNS progression. 5. Investigator-assessed ORR and detailed safety and pharmacokinetic profiling.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring during study visits and laboratory checks. Severity is determined per standard toxicity criteria. **Serious Adverse Events (SAEs):** Routine expedited reporting timeline (typically 24 hours) for events leading to hospitalization, disability, or life-threatening outcomes. **Data Monitoring Committee (DMC):** Global safety profile is regularly assessed across all cohorts by the sponsor and an external independent safety monitoring board.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Efficacy-evaluable population (patients meeting biomarker thresholds with measurable disease) and Safety-evaluable population (all enrolled patients receiving ≥ 1 dose). **Sample Size Justification:** Target enrollment of 920 patients overall, with individual expansion cohorts driven by continuous statistical evaluation of ORR threshold clearance (e.g., excluding standard-of-care ORR thresholds like 10%). **Handling of Missing Data:** Time-to-event endpoints (PFS, OS) evaluated using Kaplan-Meier methodologies with right-censoring at the last evaluable assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Centralized review of NGS results and independent central review (IRC) for all radiographic tumor assessments to ensure unbiased endpoint measurement. **Audit Trail:** Utilizing secure Electronic Data Capture (EDC) frameworks for recording and locking eCRFs.

11. Study Identifiers & Governance

Primary ID: B041932 **Registry Number:** NCT04589845 **Posting ID:** 3397838386935247 **Trial Status:** RECRUITING **Sponsor/Lead Organization:** Roche **Study Title:** Tumor-agnostic Precision Immunology and Somatic Targeting Rational for You (TAPISTRY) Platform Study **Official Regulatory Title:** Tumor-agnostic Precision Immunology and Somatic Targeting Rational for You (TAPISTRY) Phase II Platform Trial

12. Clinical Framework (Tissue-Agnostic Specific)

Primary Condition: Solid Tumors (Preferred Name: Solid Neoplasm)
MeSH Code: C04.588.550 **SNOMED ID:** 126939001 **Preferred UMLS Name:** Precision Medicine
Diagnosis Threshold: Genomic alterations or TMB-high via validated NGS assays (e.g., TMB ≥ 13 or ≥ 16 mut/Mb depending on cohort cutoffs) **Medical Methodology:** Biomarker-driven Precision Oncology
Optimization Target: Tailored assignment of genome-driven targeted therapies and immune checkpoint blockade

13. Study Procedures & Methodology

Management Pathway: Centralized or CLIA/equivalent-certified local laboratory NGS screening to triage into specific drug cohorts. **Education Protocol (HCP):** Investigators are trained on the umbrella protocol matrix matching specific mutations (e.g., ROS1, NTRK, TMB-high) to matched interventions. **Education Protocol (Patient):** Counseling regarding genomic testing and tissue-agnostic therapy implications. **Quality Audit Mechanism:** Independent Review Committee (IRC) verification of radiologic tumor size reductions.

14. Observation & Follow-Up Schedule

Total Duration: Open-ended until disease progression or unacceptable toxicity, plus long-term survival follow-up. **Onsite Visit Cadence:** Depending on cohort drug cycles (e.g., Q3W for atezolizumab). **Remote Monitoring Frequency:** Intermittent survival data collection post-treatment. **Checklist Review Interval:** Routine imaging assessments strictly every 6 to 8 weeks.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				